Healthcare Product Management

Module 4: Choosing the Right Product Archetype & Context

In healthcare, a product's success is determined as much by its organizational and technical context as by its features.

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Course Outline

By the end of Module 4, you can:

Look at a healthcare problem and determine the *right product shape, environment, and boundaries* — instead of jumping straight to features or solutions.

Up to this point, you are not designing a solution.

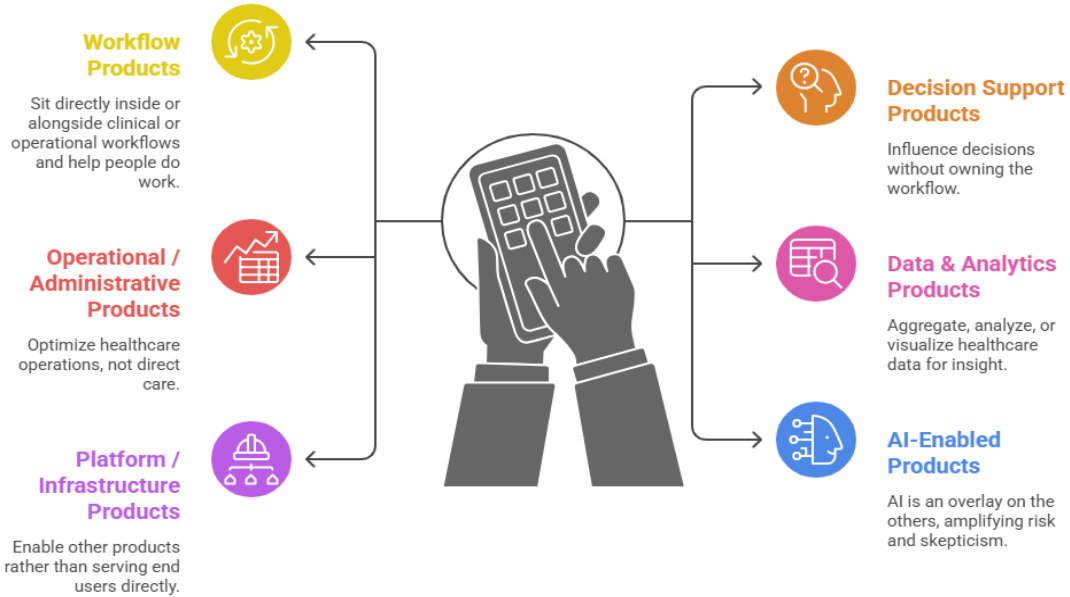
You are ***deciding what kind of product*** could responsibly exist in this healthcare system.

That decision comes *before* features, tech, or roadmaps.

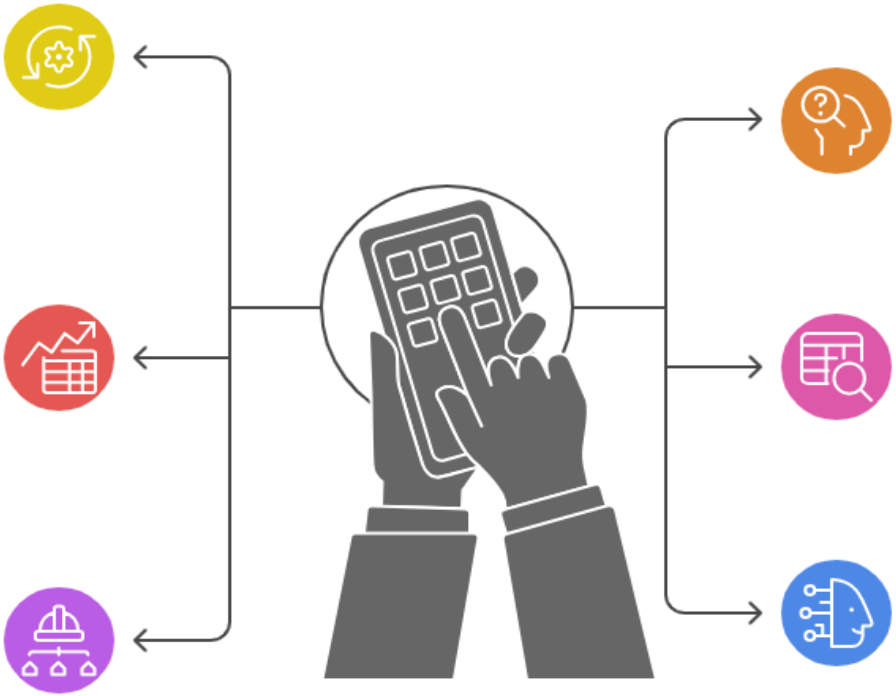
Healthcare Product Archetypes

Identifying your product's archetype clarifies your core technical constraints and user expectations.

- **Clinical Decision Support (CDS):**
  Tools that provide knowledge or patient-specific info to clinicians.
- **Workflow & Operations:**
  Tools that manage the "logistics" of care (scheduling, billing, bed management).
- **Patient-Facing/Digital Health:**
  Tools used directly by patients for monitoring or self-care.
- **Data & Analytics:**
  Infrastructure for population health, research, or financial reporting.



Healthcare Product Archetypes



Workflow Products

Sit directly inside or alongside clinical or operational workflows and help people do work.

Operational / Administrative Products

Optimize healthcare operations, not direct care.

Platform / Infrastructure Products

Enable other products rather than serving end users directly.

Decision Support Products

Influence decisions without owning the workflow.

Data & Analytics Products

Aggregate, analyze, or visualize healthcare data for insight.

AI-Enabled Products

AI is an overlay on the others, amplifying risk and skepticism.

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Diagnostics & imaging

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At-home testing
- PocDoc
- teal health

Cancer prognosis
- ATARAXIS

Cardiac diagnostics
- anumana

Interventional imaging
- MENDAERA

Workflow automation
- gestalt
- Rad AI

Administrative operations

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Value-based care contracting
- Arbital Health

Revenue cycle management
- Autonomize
- Infinitus
- Mandolin
- plenful
- sohar

Governance & risk management
- 1m
- HealthEx
- Qualified Health

Supply chain
- AssistIQ

Patient communication & engagement
- Assort Health
- ELLIPSIS HEALTH
- kouper
- hyro

Health insurance
- Fortuna Health
- THATCH

Drug discovery & development
- PERSIST AI
- PHASE V
- VARIATIONAL AI

Senior care
- August
- inspiren
- SafelyYou

Clinical workflow automation

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Ambient documentation
- Ambience
- Freed
- Nabla

Care delivery infrastructure
- CalmWave
- hellocare.ai

Chart review & abstraction
- CARTA HEALTHCARE
- LAYER HEALTH

Clinical intelligence & support
- ATROPOS HEALTH
- guidehealth
- Hippocratic AI
- xund

Care delivery

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Neurology
- Isaac health
- SEVARO

Nutrition
- Fay
- Nourish

Behavioral health
- Frontera

Women's health
- DELFINA

EMS
- Tele911

In-home care
- sprinter health

Longevity clinics
- Function
- NEKO

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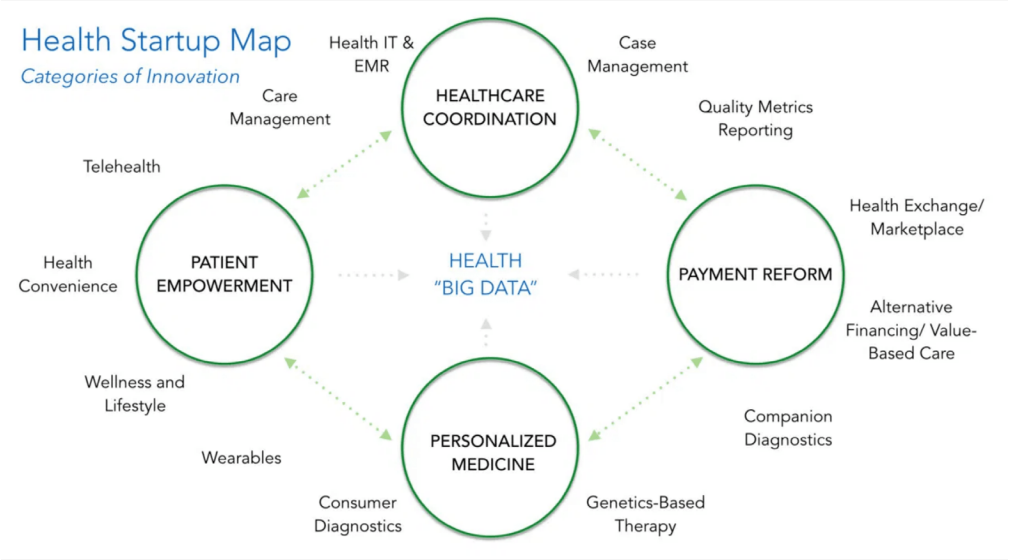
Note: Companies are private as of 10/7/2025.
Categories are not mutually exclusive.

Hospital vs. Vendor vs. Startup Contexts

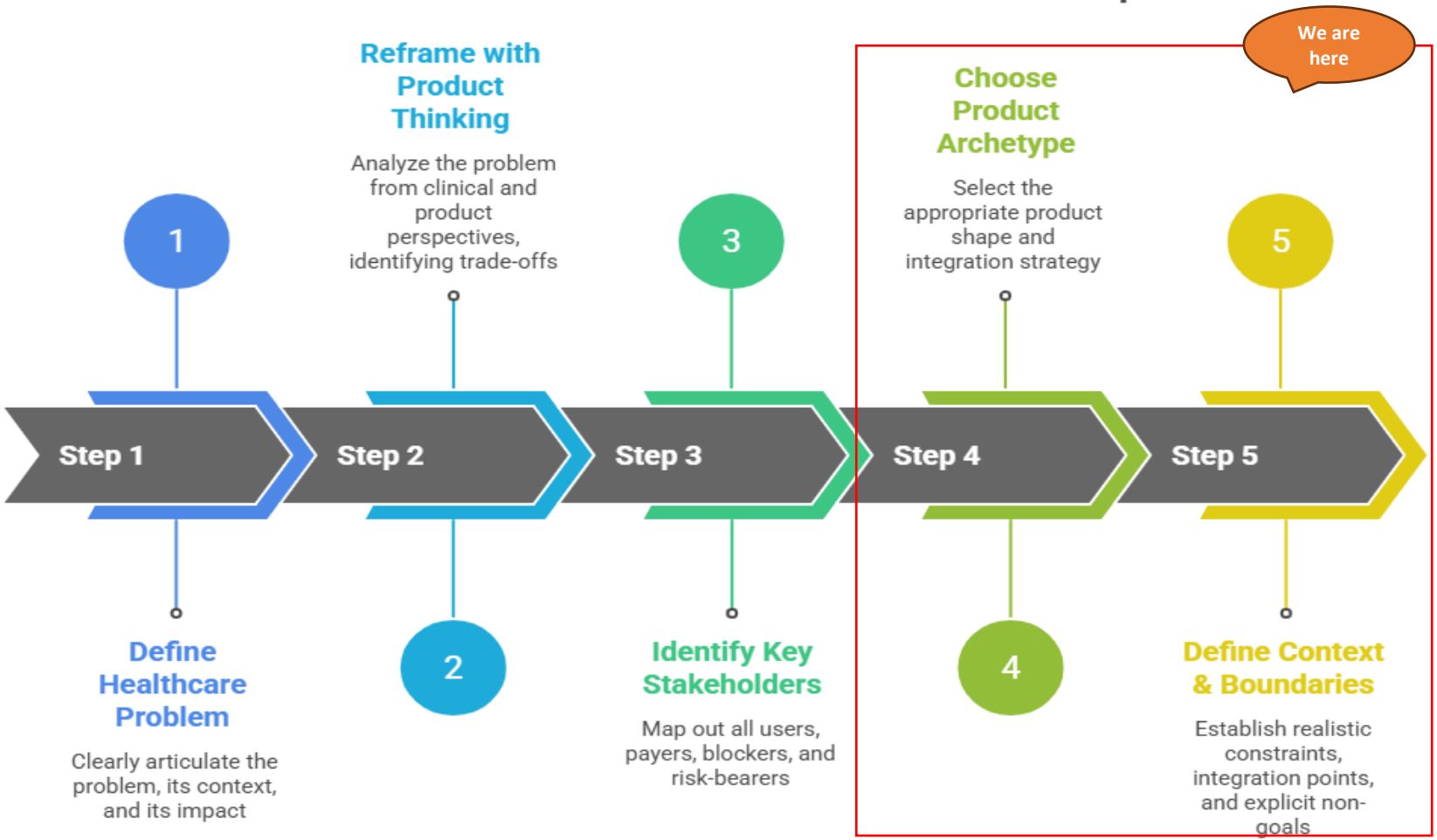
Where you build determines your speed, your constraints, and your definition of success.

- **Hospital/Health System:**
  High access to users, but slow procurement and complex legacy IT.
- **Established Vendor:**
  Massive scale and trust, but rigid roadmaps and high technical debt.
- **HealthTech Startup:**
  High speed and innovation, but limited trust and difficult integration hurdles.



Artifact Prep: Select the organizational setting for your concept and justify why it's the best fit.

Foundation Workflow for Healthcare Product Development



Five Questions That Define Product Reality

1. Where does this product have to live to be used at all?

2. Who actually owns the risk if this product is wrong?

- Clinician, organization, or vendor?
- Liability concerns?

3. Who must say "yes" for this to exist—and who can kill it?

- Key approvers?
- Hidden blockers?

4. What data must already exist for this to work?

- Reliable, accessible data?
- Technical limitations?

5. What must this product explicitly NOT try to do?

- Scope boundaries?
- Out-of-scope risks?

The Integration Reality: EHR-Integrated vs. Standalone

Integration is not a "feature"; it is a fundamental strategic choice that dictates adoption.

EHR-Integrated Model

- High adoption: Clinicians don't leave their existing workflow.
- High technical complexity and longer deployment cycles.

Standalone / Web-Based

- Faster to build and deploy with fewer technical hurdles.
- Risks being ignored as "one more login" in a busy environment.

Judgment Call: The "Tab Fatigue" Test

Is the specific value of your tool high enough to justify a standalone login? Clinicians suffer from "tab fatigue" just as much as "alert fatigue." The "App Orchard" model can bridge this gap.

Workflow, Data, and Compliance Constraints

Every healthcare context comes with a pre-defined set of non-negotiable boundaries.

Workflow Constraints

Does the tool fit into the physical and temporal reality of the care setting (e.g., ER vs. Home)?

Data Constraints

Where does the data live, who owns it, and how does it move (HL7, FHIR, proprietary APIs)?

Compliance Constraints

Is this a Class II medical device? Does it require SOC2, HIPAA, or specific regional certifications?

Artifact Prep: Constraint Mapping

List the top three constraints (Workflow, Data, or Compliance) for your chosen context. Emphasize the parameters you must design within.

Common Misalignment Traps

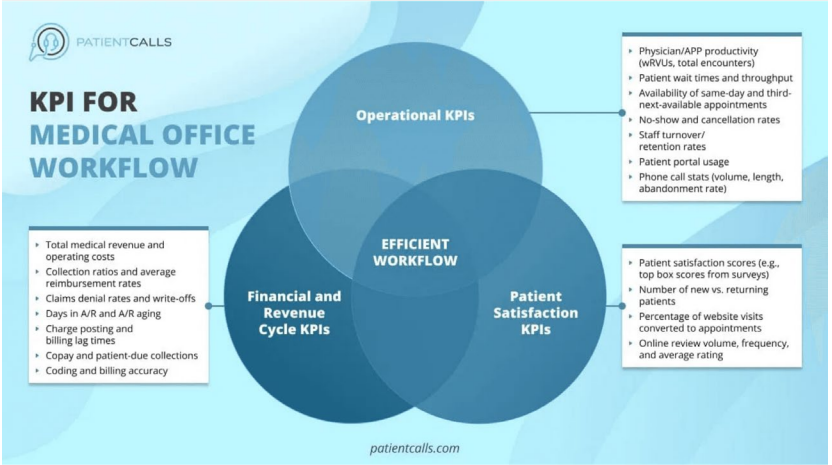
Most healthcare product failures start with a mismatch between the archetype and the context.

- **The "Over-Engineered" Trap:**
- Building a complex CDS tool for a setting that just needs simple workflow automation.
- **The "Integration Blindness" Trap:**
- Building a standalone tool for a workflow that happens 100% inside the EHR.
- **The "Regulatory Surprise" Trap:**
- Treating a clinical tool like a consumer app and hitting FDA hurdles too late.
- **The "Incentive Mismatch":**
- Building for a "Value-Based" context in a "Fee-for-Service" hospital.

Defining Explicit Non-Goals

In a resource-constrained environment, what you *won't* do is as important as what you will.

- **Non-goals** prevent "scope creep" and keep the team focused on the core problem.
- **Example: "We will NOT integrate with billing systems in Version 1."**
- **Example: "This tool is NOT for pediatric patients."**



Artifact Prep: Define at least two explicit non-goals for your product concept.

Artifact Checkpoint: What You Should Have So Far

Your artifact should now have a clear "Product Context & Constraints" section.

- ☑ **Product Archetype**
  e.g., "Workflow & Operations tool for ER bed management".
- ☑ **Organizational Setting**
  e.g., "Built as an internal tool within a large academic medical center".
- ☑ **Constraints**
  Clear list of data (FHIR), workflow (mobile-first), and compliance (HIPAA) needs.
- ☑ **Non-Goals**
  Explicit list of what the product will NOT attempt to solve.

Summary: Choosing Your Battleground

Successful PMs don't just build great products; they choose the right context to build them in.

Archetypes

Archetypes provide the high-level technical roadmap and define the core regulatory and user expectations for your product.

Context

Context provides the organizational rules; where you build determines your speed, constraints, and definition of success.

Constraints

Constraints are not problems to be solved, but parameters to be designed within—respecting the reality of workflow and data.

Next Module: Developing the Value Proposition and MVP Strategy